

Name : Ankit(26Y/M)

Date : 29 Mar, 2026

Test Asked : Aarogyam Full Health Checkup With Vitamins, Phos +
1 Other

Report Status : Complete Report



6^{STEP} quality control to ensure 100% report accuracy



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DOCON TECHNOLOGIES PRIVATE LIMITED, Office No.208, 209, 210 Second floor,
A wing, 'Dattani Plaza', Near East West Industrial Estate, Safed Pool, Saki Naka,
Andheri (East), Mumbai - 400072

Patient Name : ANKIT(26Y/M)
Referred By : SELF
Home Collection : A-37 PRATIMA BHAWAN SECTOR 26 A BLOCK NOIDA NOIDA-01 NOIDA

Tests Done : AAROGYAM FULL HEALTH CHECKUP
WITH VITAMINS,PHOS,ESR

Report Availability Summary

Note: Please refer to the table below for status of your tests.

18 Ready 0 Ready with Cancellation 0 Processing 0 Cancelled in Lab

TEST DETAILS	REPORT STATUS
ERYTHROCYTE SEDIMENTATION RATE (ESR)	Ready
PHOSPHOROUS	Ready
AAROGYAM FULL HEALTH CHECKUP WITH VITAMINS	Ready
FASTING BLOOD SUGAR(GLUCOSE)	Ready
CHLORIDE	Ready
25-OH VITAMIN D (TOTAL)	Ready
HIGH SENSITIVITY C-REACTIVE PROTEIN (HS-CRP)	Ready
IRON	Ready
Lipoprotein (a) [Lp(a)]	Ready
SODIUM	Ready
VITAMIN B-12	Ready
COMPLETE URINE ANALYSIS	Ready
HBA PROFILE	Ready
HEMOGRAM - 6 PART (DIFF)	Ready
LIVER FUNCTION TESTS	Ready
KIDPRO	Ready
LIPID PROFILE	Ready
T3-T4-USTSH	Ready
APOLIPROTEIN RATIO	Ready

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Tests Outside Reference Range

Note: Please refer to the table below for tests outside reference range.

Test Name	Observed Value	Units	Bio. Ref. Interval.
CARDIAC RISK MARKERS			
APO B / APO A1 RATIO (APO B/A1)	1.3	Ratio	0.40 - 1.26
HIGH SENSITIVITY C-REACTIVE PROTEIN (HS-CRP)	3.16	mg/L	< 3
COMPLETE HEMOGRAM			
BASOPHILS - ABSOLUTE COUNT	0.01	X 10 ³ / µL	0.02 - 0.1
MEAN CORPUSCULAR HEMOGLOBIN(MCH)	26	pq	27.0-32.0
MEAN CORPUSCULAR VOLUME(MCV)	81.3	fL	83.0-101.0
PLATELET DISTRIBUTION WIDTH(PDW)	9.5	fL	9.6-15.2
PLATELET TO LARGE CELL RATIO(PLCR)	17.8	%	19.7-42.4
RED CELL DISTRIBUTION WIDTH - SD(RDW-SD)	37.2	fL	39-46
TOTAL RBC	5.76	X 10 ⁶ /µL	4.5-5.5
COMPLETE URINE ANALYSIS			
SPECIFIC GRAVITY	< 1.003	-	1.003-1.030
IRON DEFICIENCY			
IRON	50	µg/dL	65 - 175
LIPID			
HDL / LDL RATIO	0.21	Ratio	> 0.40
HDL CHOLESTEROL - DIRECT	36	mg/dL	40-60
LDL / HDL RATIO	4.7	Ratio	1.5-3.5
LDL CHOLESTEROL - DIRECT	165.68	mg/dL	< 100
NON-HDL CHOLESTEROL	184.19	mg/dL	< 160
TC/ HDL CHOLESTEROL RATIO	6.2	Ratio	3 - 5
TOTAL CHOLESTEROL	220	mg/dL	< 200
TRIG / HDL RATIO	4.63	Ratio	< 3.12
TRIGLYCERIDES	164	mg/dL	< 150
LIVER			
ALKALINE PHOSPHATASE	129.1	U/L	45-129
BILIRUBIN (INDIRECT)	1.21	mg/dL	0-0.9
BILIRUBIN - TOTAL	1.43	mg/dL	0.3-1.2
BILIRUBIN -DIRECT	0.22	mg/dL	0 - 0.20
VITAMIN			
25-OH VITAMIN D (TOTAL)	7.4	ng/mL	30-100
VITAMIN B-12	101	pg/mL	197-771

Disclaimer: The above listed is the summary of the parameters with values outside the BRI. For detailed report values, parameter correlation and clinical interpretation, kindly refer to the same in subsequent pages.



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Report Released on (RRT) : 29 Mar 2026 16:29
Sample Type | Barcode : EDTA Whole Blood | FF874681

TEST NAME	TECHNOLOGY	VALUE	UNITS
HbA1c	H.P.L.C	5.5	%

Bio. Ref. Interval. :

As per ADA Guidelines

Below 5.7% : Normal
5.7% - 6.4% : Prediabetic
>=6.5% : Diabetic

Guidance For Known Diabetics

Below 6.5% : Good Control
6.5% - 7% : Fair Control
7.0% - 8% : Unsatisfactory Control
>8% : Poor Control

Method : Fully Automated H.P.L.C method

AVERAGE BLOOD GLUCOSE (ABG) CALCULATED 111 mg/dL

Bio. Ref. Interval. :

90 - 120 mg/dl : Good Control
121 - 150 mg/dl : Fair Control
151 - 180 mg/dl : Unsatisfactory Control
> 180 mg/dl : Poor Control

Method : Derived from HbA1c values

Please correlate with clinical conditions.

Tests Done : AAROYAM FULL HEALTH CHECKUP WITH
VITAMINS,ESR,PHOSPHOROUS

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd-NABL
accredited, NABL certificate no: MC-6252



Dr.Sneha Singh
MD(Path)



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TEST NAME	TECHNOLOGY	VALUE	UNITS
ERYTHROCYTE SEDIMENTATION RATE (ESR) Bio. Ref. Interval. :-	MODIFIED WESTERGREN	12	mm / hr

<50 yr : Male: 0-15 mm/hr Female: 0-20 mm/hr
>50 yr : Male: 0-20 mm/hr Female: 0-30 mm/hr
Children: <=10 mm/hr

Clinical Significance:

- An erythrocyte sedimentation rate (ESR) is a blood test that can rise if you have inflammation in your body. Its also used as a marker to monitor prognosis of an existing inflammatory/infective condition.
- Inflammation is your immune systems response to injury, infection, and many types of conditions, including immune system disorders, certain cancers and blood disorders.
- A high ESR test result may be from a condition that causes inflammation, such as: Arteritis, Arthritis, Systemic vasculitis, Polymyalgia rheumatica, Inflammatory bowel disease, Kidney disease, Infections like Tuberculosis etc, Rheumatoid arthritis and other autoimmune diseases, Heart disease, Certain cancers and many other Conditions.
- A low ESR test result may be caused by conditions such as: A blood disorder, such as: Polycythemia, Sickle cell disease (SCD), Leukocytosis, Heart failure, Certain kidney and liver problems etc.
- Certain physiological conditions also affect ESR results, these include : Pregnancy, menstrual cycle, ageing, obesity, drinking alcohol regularly, and exercise, Certain medicines and supplements also can affect ESR results.
- Hence Its always suggested to interpret ESR results in conjunction with Clinical History and other findings.

References :

<https://medlineplus.gov/lab-tests/erythrocyte-sedimentation-rate-esr/>

Please correlate with clinical conditions.

Method:- MODIFIED WESTERGREN

Tests Done : AAROGRAM FULL HEALTH CHECKUP WITH
VITAMINS,ESR,PHOSPHOROUS

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TEST NAME	METHODOLOGY	VALUE	UNITS	Bio. Ref. Interval.
HEMOGLOBIN	SLS-Hemoglobin Method	15	g/dL	13.0-17.0
Hematocrit (PCV)	CPH Detection	46.8	%	40.0-50.0
Total RBC	HF & EI	5.76	X 10⁶/μL	4.5-5.5
Mean Corpuscular Volume (MCV)	Calculated	81.3	fL	83.0-101.0
Mean Corpuscular Hemoglobin (MCH)	Calculated	26	pq	27.0-32.0
Mean Corp.Hemo. Conc (MCHC)	Calculated	32.1	g/dL	31.5-34.5
Red Cell Distribution Width - SD (RDW-SD)	Calculated	37.2	fL	39-46
Red Cell Distribution Width (RDW - CV)	Calculated	12.7	%	11.6-14
RED CELL DISTRIBUTION WIDTH INDEX (RDWI)	Calculated	179.3	-	*Refer Note below
MENTZER INDEX	Calculated	14.1	-	*Refer Note below
TOTAL LEUCOCYTE COUNT (WBC)	HF & FC	7.49	X 10 ³ / μL	4.0 - 10.0
DIFFERENTIAL LEUCOCYTE COUNT				
Neutrophils Percentage	Flow Cytometry	67.6	%	40-80
Lymphocytes Percentage	Flow Cytometry	21.9	%	20-40
Monocytes Percentage	Flow Cytometry	4.5	%	2-10
Eosinophils Percentage	Flow Cytometry	5.9	%	1-6
Basophils Percentage	Flow Cytometry	0.1	%	0-2
Immature Granulocyte Percentage (IG%)	Flow Cytometry	0.1	%	0-0.5
Nucleated Red Blood Cells %	Flow Cytometry	0.01	%	0.0-5.0
ABSOLUTE LEUCOCYTE COUNT				
Neutrophils - Absolute Count	Calculated	5.06	X 10 ³ / μL	2.0-7.0
Lymphocytes - Absolute Count	Calculated	1.64	X 10 ³ / μL	1.0-3.0
Monocytes - Absolute Count	Calculated	0.34	X 10 ³ / μL	0.2 - 1.0
Basophils - Absolute Count	Calculated	0.01	X 10³ / μL	0.02 - 0.1
Eosinophils - Absolute Count	Calculated	0.44	X 10 ³ / μL	0.02 - 0.5
Immature Granulocytes (IG)	Calculated	0.01	X 10 ³ / μL	0-0.3
Nucleated Red Blood Cells	Calculated	0.01	X 10 ³ / μL	0.0-0.5
PLATELET COUNT	HF & EI	259	X 10 ³ / μL	150-410
Mean Platelet Volume (MPV)	Calculated	9.2	fL	6.5-12
Platelet Distribution Width (PDW)	Calculated	9.5	fL	9.6-15.2
Platelet to Large Cell Ratio (PLCR)	Calculated	17.8	%	19.7-42.4
Plateletcrit (PCT)	Calculated	0.24	%	0.19-0.39

Remarks : Alert!!! Predominantly normocytic normochromic with ovalocytes. Platelets:Appear adequate in smear.

*Note - Mentzer index (MI), RDW-CV and RDWI are hematological indices to differentiate between Iron Deficiency Anemia (IDA) and Beta Thalassemia Trait (BTT). MI >13, RDWI >220 and RDW-CV >14 more likely to be IDA. MI <13, RDWI <220, and RDW-CV <14 more likely to be BTT. Suggested Clinical correlation. BTT to be confirmed with HB electrophoresis if clinically indicated.

Method : Fully automated bidirectional analyser (6 Part Differential SYSMEX XN-1000)

(Reference : *FC- flowcytometry, *HF- hydrodynamic focussing, *EI- Electric Impedence, *Hb- hemoglobin, *CPH- Cumulative pulse height)

Tests Done : AAROGYAM FULL HEALTH CHECKUP WITH
VITAMINS,ESR,PHOSPHOROUS

Sneha

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd

Dr.Sneha Singh
MD(Path)



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Sample Type | Barcode : SERUM | FA445652

TEST NAME	TECHNOLOGY	VALUE	UNITS
25-OH VITAMIN D (TOTAL) Bio. Ref. Interval. :-	E.C.L.I.A	7.4	ng/mL

Deficiency : ≤ 20 ng/ml || Insufficiency : 21-29 ng/ml
Sufficiency : ≥ 30 ng/ml || Toxicity : > 100 ng/ml

Clinical Significance:

Vitamin D is a fat soluble vitamin that has been known to help the body absorb and retain calcium and phosphorous; both are critical for building bone health.

Decrease in vitamin D total levels indicate inadequate exposure of sunlight, dietary deficiency, nephrotic syndrome.

Increase in vitamin D total levels indicate Vitamin D intoxication.

Specifications: Precision: Intra assay (%CV):9.20%, Inter assay (%CV):8.50%

Kit Validation Reference : Holick M. Vitamin D the underappreciated D-Lightful hormone that is important for Skeletal and cellular health Curr Opin Endocrinol Diabetes 2002;9(1)87-98.

Please correlate with clinical conditions.

Method:- Fully Automated Electrochemiluminescence Competitive Immunoassay

Tests Done : AAROGRAM FULL HEALTH CHECKUP WITH
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TEST NAME	TECHNOLOGY	VALUE	UNITS
APOLIPOPROTEIN - A1 (APO-A1) Bio. Ref. Interval. : Male : 86 - 152 Female : 94 - 162 Method : FULLY AUTOMATED RATE IMMUNOTURBIDIMETRY - BECKMAN COULTER	IMMUNOTURBIDIMETRY	89	mg/dL
APOLIPOPROTEIN - B (APO-B) Bio. Ref. Interval. : Male : 56 - 145 Female : 53 - 138 Method : FULLY AUTOMATED RATE IMMUNOTURBIDIMETRY - BECKMAN COULTER	IMMUNOTURBIDIMETRY	116	mg/dL
APO B / APO A1 RATIO (APO B/A1) Bio. Ref. Interval. : Male : 0.40 - 1.26 Female : 0.38 - 1.14 Clinical Significance : • Apolipoprotein B is a more potent and independent predictor of Coronary artery disease (CAD) than LDL Cholesterol. • Apolipoprotein A1 is one of the apoproteins of HDL and is inversely related to risk of CAD. • The Apolipoprotein studies help in monitoring risk of restenosis in patients with myocardial infarction, Coronary bypass surgery etc. • An increased ratio of Apo B to A1 beyond the defined normal range is indicative of CAD risk. • All results have to be interpreted in Conjunction with clinical history and other findings. Method : Derived from serum Apo A1 and Apo B values	CALCULATED	1.3	Ratio
Please correlate with clinical conditions.			

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TEST NAME	TECHNOLOGY	VALUE	UNITS
VITAMIN B-12 Bio. Ref. Interval. :-	E.C.L.I.A	101	pg/mL

Normal: 197-771 pg/ml

Clinical significance :

Vitamin B12 or cyanocobalamin, is a complex corrinoid compound found exclusively from animal dietary sources, such as meat, eggs and milk. It is critical in normal DNA synthesis, which in turn affects erythrocyte maturation and in the formation of myelin sheath. Vitamin-B12 is used to find out neurological abnormalities and impaired DNA synthesis associated with macrocytic anemias. For diagnostic purpose, results should always be assessed in conjunction with the patients medical history, clinical examination and other findings.

Specifications: Intra assay (%CV):2.6%, Inter assay (%CV):2.3 %

Kit Validation Reference : Thomas L.Clinical laborator Diagnostics : Use and Assessment of Clinical laboratory Results 1st Edition,TH Books-Verl-Ges,1998:424-431

Please correlate with clinical conditions.

Method:- Fully Automated Electrochemiluminescence Compititive Immunoassay

Tests Done : AAROGRAM FULL HEALTH CHECKUP WITH
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TEST NAME	TECHNOLOGY	VALUE	UNITS
Lipoprotein (a) [Lp(a)] Bio. Ref. Interval. :-	IMMUNOTURBIDIMETRY	10.7	mg/dL

Adults : < 30.0 mg/dl

Clinical Significance:

Determination of LPA may be useful to guide management of individuals with a family history of CHD or with existing disease. The levels of LPA in the blood depends on genetic factors; The range of variation in a population is relatively large and hence for diagnostic purpose, results should always be assessed in conjunction with the patient's medical history, clinical examination and other findings.

Specifications:

Precision %CV :- Intra assay %CV- 4.55% , Inter assay %CV-0.86 %

Kit Validation Reference:

Tietz NW, Clinical Guide to Laboratory Tests Philadelphia WB. Saunders 1995 : 442-444

Please correlate with clinical conditions.

Method:- LATEX ENHANCED IMMUNOTURBIDIMETRY

Tests Done : AAROGYAM FULL HEALTH CHECKUP WITH
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TEST NAME	TECHNOLOGY	VALUE	UNITS
HIGH SENSITIVITY C-REACTIVE PROTEIN (HS-CRP) Bio. Ref. Interval. :-	IMMUNOTURBIDIMETRY	3.16	mg/L

< 1.00 - Low Risk
1.00 - 3.00 - Average Risk
>3.00 - 10.00 - High Risk
> 10.00 - Possibly due to Non-Cardiac Inflammation

Disclaimer: Persistent unexplained elevation of HSCRP >10 should be evaluated for non-cardiovascular etiologies such as infection, active arthritis or concurrent illness.

Clinical significance:

High sensitivity C- reactive Protein (HSCRP) can be used as an independent risk marker for the identification of Individuals at risk for future cardiovascular Disease. A coronary artery disease risk assessment should be based on the average of two hs-CRP tests, ideally taken two weeks apart.

Kit Validation Reference:

- 1.Clinical management of laboratory data in medical practice 2003-3004, 207(2003).
- 2.Tietz : Textbook of Clinical Chemistry and Molecular diagnostics :Second edition :Chapter 47:Page no.1507- 1508.

Please correlate with clinical conditions.

Method:- FULLY AUTOMATED LATEX AGGLUTINATION – BECKMAN COULTER

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval
TOTAL CHOLESTEROL	PHOTOMETRY	220	mg/dL	< 200
HDL CHOLESTEROL - DIRECT	PHOTOMETRY	36	mg/dL	40-60
LDL CHOLESTEROL - DIRECT	PHOTOMETRY	165.68	mg/dL	< 100
TRIGLYCERIDES	PHOTOMETRY	164	mg/dL	< 150
TC/ HDL CHOLESTEROL RATIO	CALCULATED	6.2	Ratio	3 - 5
TRIG / HDL RATIO	CALCULATED	4.63	Ratio	< 3.12
LDL / HDL RATIO	CALCULATED	4.7	Ratio	1.5-3.5
HDL / LDL RATIO	CALCULATED	0.21	Ratio	> 0.40
NON-HDL CHOLESTEROL	CALCULATED	184.19	mg/dL	< 160
VLDL CHOLESTEROL	CALCULATED	32.86	mg/dL	5 - 40

Please correlate with clinical conditions.

Method :

CHOL - Cholesterol Oxidase, Esterase, Peroxidase
HCHO - Direct Enzymatic Colorimetric
LDL - Direct Measure
TRIG - Enzymatic, End Point
TC/H - Derived from serum Cholesterol and Hdl values
TRI/H - Derived from TRIG and HDL Values
LDL/ - Derived from serum HDL and LDL Values
HD/LD - Derived from HDL and LDL values.
NHDH - Derived from serum Cholesterol and HDL values
VLDL - Derived from serum Triglyceride values

*REFERENCE RANGES AS PER NCEP ATP III GUIDELINES:

TOTAL CHOLESTEROL	(mg/dl)	HDL	(mg/dl)	LDL	(mg/dl)	TRIGLYCERIDES	(mg/dl)
DESIRABLE	<200	LOW	<40	OPTIMAL	<100	NORMAL	<150
BORDERLINE HIGH	200-239	HIGH	>60	NEAR OPTIMAL	100-129	BORDERLINE HIGH	150-199
HIGH	>240			BORDERLINE HIGH	130-159	HIGH	200-499
				HIGH	160-189	VERY HIGH	>500
				VERY HIGH	>190		

Alert !!! 10-12 hours fasting is mandatory for lipid parameters. If not, values might fluctuate.

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval
ALKALINE PHOSPHATASE	PHOTOMETRY	129.1	U/L	45-129
BILIRUBIN - TOTAL	PHOTOMETRY	1.43	mg/dL	0.3-1.2
BILIRUBIN -DIRECT	PHOTOMETRY	0.22	mg/dL	0 - 0.20
BILIRUBIN (INDIRECT)	CALCULATED	1.21	mg/dL	0-0.9
GAMMA GLUTAMYL TRANSFERASE (GGT)	PHOTOMETRY	19.69	U/L	< 55
ASPARTATE AMINOTRANSFERASE (SGOT)	PHOTOMETRY	18.91	U/L	< 35
ALANINE TRANSAMINASE (SGPT)	PHOTOMETRY	23.28	U/L	< 45
SGOT / SGPT RATIO	CALCULATED	0.81	Ratio	< 2
PROTEIN - TOTAL	PHOTOMETRY	7.25	gm/dL	5.7-8.2
ALBUMIN - SERUM	PHOTOMETRY	4.64	gm/dL	3.2-4.8
SERUM GLOBULIN	CALCULATED	2.61	gm/dL	2.5-3.4
SERUM ALB/GLOBULIN RATIO	CALCULATED	1.78	Ratio	0.9 - 2

Please correlate with clinical conditions.

Method :

ALKP - Modified IFCC method
BILT - Diazonium salt DPD method
BILD - Diazonium salt DPD method
BILI - Derived from serum Total and Direct Bilirubin values
GGT - Modified IFCC method
SGOT - IFCC* Without Pyridoxal Phosphate Activation
SGPT - IFCC* Without Pyridoxal Phosphate Activation
OT/PT - Derived from SGOT and SGPT values.
PROT - Biuret Method
SALB - Albumin Bcg¹method (Colorimetric Assay Endpoint)
SEGB - DERIVED FROM SERUM ALBUMIN AND PROTEIN VALUES
A/GR - Derived from serum Albumin and Protein values

Tests Done : AAROYAM FULL HEALTH CHECKUP WITH
VITAMINS,ESR,PHOSPHOROUS

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd-NABL
accredited, NABL certificate no: MC-6252



Dr.Sneha Singh
MD(Path)



DOCON TECHNOLOGIES PRIVATE LIMITED, Office No.208, 209, 210 Second floor,
A wing, 'Dattani Plaza', Near East West Industrial Estate, Safed Pool, Saki Naka,
Andheri (East), Mumbai - 400072

Patient Name : ANKIT(26Y/M)
Referred By : SELF
Home Collection : A-37 PRATIMA BHAWAN SECTOR 26 A BLOCK NOIDA NOIDA-01
NOIDA

Sample Collected on (SCT) : 29 Mar 2026 10:41
Sample Received on (SRT) : 29 Mar 2026 12:33
Report Released on (RRT) : 29 Mar 2026 17:27
Sample Type | Barcode : SERUM | FA445652

TEST NAME	TECHNOLOGY	VALUE	UNITS
PHOSPHOROUS	PHOTOMETRY	3.81	mg/dL

Bio. Ref. Interval. :

Adults : 2.4 - 5.1 mg/dL

Children : 4.0 - 7.0 mg/dL

Clinical Significance:

In plasma and serum the majority of phosphate exists in the inorganic form (Pi), approximately 15% bound to protein and the remainder in complexes and free forms. Serum phosphate concentrations are dependent on diet and variation in the secretion of hormones such as Parathyroid Hormone (PTH).

Specifications:

Precision %CV :- Intra assay %CV- 1.55% , Inter assay %CV-2.99% , Sensitivity:-0.10 mmol/L

Kit Validation Reference:

Young DS. Effects of drugs on clinical laboratory tests, 5th ed. AACC Press, 2000.

Method : UNREDUCED PHOSPHOMOLYBDATE METHOD

Please correlate with clinical conditions.

Tests Done : AAROYAM FULL HEALTH CHECKUP WITH
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TEST NAME	TECHNOLOGY	VALUE	UNITS
CHLORIDE	I.S.E - INDIRECT	102.3	mmol/L
Bio. Ref. Interval. : ADULTS: 98-107 MMOL/L			
Clinical Significance : An increased level of blood chloride (called hyperchloremia) usually indicates dehydration, but can also occur with other problems that cause high blood sodium, such as Cushing syndrome or kidney disease. Hyperchloremia also occurs when too much base is lost from the body (producing metabolic acidosis) or when a person hyperventilates (causing respiratory alkalosis). A decreased level of blood chloride (called hypochloremia) occurs with any disorder that causes low blood sodium. Hypochloremia also occurs with congestive heart failure, prolonged vomiting or gastric suction, Addison disease, emphysema or other chronic lung diseases (causing respiratory acidosis), and with loss of acid from the body (called metabolic alkalosis).			
Method : ION SELECTIVE ELECTRODE - INDIRECT			
SODIUM	I.S.E - INDIRECT	139.9	mmol/L
Bio. Ref. Interval. : Adults: 136-145 mmol/l			
Method : ION SELECTIVE ELECTRODE - INDIRECT			
Please correlate with clinical conditions.			

Tests Done : AAROYAM FULL HEALTH CHECKUP WITH
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Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd-NABL
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MD(Path)

Processed At :
D-79, 3rd Floor, Sector-63,
Gautam Budh Nagar, Noida,
UP-201301



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TEST NAME	TECHNOLOGY	VALUE	UNITS
IRON	PHOTOMETRY	50	µg/dL
Bio. Ref. Interval. : Male : 65 - 175 Female : 50 - 170			
Method : Ferrozine method without deproteinization			
Please correlate with clinical conditions.			

Tests Done : AAROYAM FULL HEALTH CHECKUP WITH
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Sample Type | Barcode : SERUM | FA445652

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval
BLOOD UREA NITROGEN (BUN)	PHOTOMETRY	10.6	mg/dL	7.94 - 20.07
CREATININE - SERUM	PHOTOMETRY	0.79	mg/dL	0.72-1.18
BUN / Sr.CREATININE RATIO	CALCULATED	13.42	Ratio	9:1-23:1
UREA (CALCULATED)	CALCULATED	22.68	mg/dL	Adult : 17-43
UREA / SR.CREATININE RATIO	CALCULATED	28.71	Ratio	< 52
CALCIUM	PHOTOMETRY	9.7	mg/dL	8.8-10.6
URIC ACID	PHOTOMETRY	6.7	mg/dL	4.2 - 7.3

Please correlate with clinical conditions.

Method :

BUN - Kinetic UV Assay.
SCRE - Creatinine Enzymatic Method
B/CR - Derived from serum Bun and Creatinine values
UREAC - Derived from BUN Value.
UR/CR - Derived from UREA and Sr.Creatinine values.
CALC - Arsenazo III Method, End Point.
URIC - Uricase / Peroxidase Method

Tests Done : AAROGYAM FULL HEALTH CHECKUP WITH
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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL TRIIODOTHYRONINE (T3)	E.C.L.I.A	136	ng/dL	80-200
TOTAL THYROXINE (T4)	E.C.L.I.A	8.78	µg/dL	4.8-12.7
TSH - ULTRASENSITIVE	E.C.L.I.A	1.07	µIU/mL	0.54-5.30

Comments : ***

The Biological Reference Ranges is specific to the age group. Kindly correlate clinically.

Method :

T3,T4 - Fully Automated Electrochemiluminescence Compititive Immunoassay
USTSH - Fully Automated Electrochemiluminescence Sandwich Immunoassay

Disclaimer :Results should always be interpreted using the reference range provided by the laboratory that performed the test. Different laboratories do tests using different technologies, methods and using different reagents which may cause difference. In reference ranges and hence it is recommended to interpret result with assay specific reference ranges provided in the reports. To diagnose and monitor therapy doses, it is recommended to get tested every time at the same Laboratory.

Tests Done : AAROGYAM FULL HEALTH CHECKUP WITH
VITAMINS,ESR,PHOSPHOROUS

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd-NABL
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Report Released on (RRT) : 29 Mar 2026 17:27
Sample Type | Barcode : SERUM | FA445652

TEST NAME	TECHNOLOGY	VALUE	UNITS
EST. GLOMERULAR FILTRATION RATE (eGFR) Bio. Ref. Interval. :-	CALCULATED	126	mL/min/1.73 m2

> = 90 : Normal
60 - 89 : Mild Decrease
45 - 59 : Mild to Moderate Decrease
30 - 44 : Moderate to Severe Decrease
15 - 29 : Severe Decrease
<15 : Kidney Failure

Clinical Significance

The normal serum creatinine reference interval does not necessarily reflect a normal GFR for a patient. Because mild and moderate kidney injury is poorly inferred from serum creatinine alone. Thus, it is recommended for clinical laboratories to routinely estimate glomerular filtration rate (eGFR), a "gold standard" measurement for assessment of renal function, and report the value when serum creatinine is measured for patients 18 and older, when appropriate and feasible. It cannot be measured easily in clinical practice, instead, GFR is estimated from equations using serum creatinine, age, race and sex. This provides easy to interpret information for the doctor and patient on the degree of renal impairment since it approximately equates to the percentage of kidney function remaining. Application of CKD-EPI equation together with the other diagnostic tools in renal medicine will further improve the detection and management of patients with CKD.

Reference

Levey AS, Stevens LA, Schmid CH, Zhang YL, Castro AF, 3rd, Feldman HI, et al. A new equation to estimate glomerular filtration rate. Ann Intern Med. 2009;150(9):604-12.

Please correlate with clinical conditions.

Method:- 2021 CKD EPI Creatinine Equation

Tests Done : AAROGYAM FULL HEALTH CHECKUP WITH
VITAMINS,ESR,PHOSPHOROUS

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd-NABL
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Patient Name : ANKIT(26Y/M)
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NOIDA

Sample Collected on (SCT) : 29 Mar 2026 10:41
Sample Received on (SRT) : 29 Mar 2026 12:37
Report Released on (RRT) : 29 Mar 2026 13:34
Sample Type | Barcode : FLUORIDE PLASMA | FJ357675

TEST NAME	TECHNOLOGY	VALUE	UNITS
FASTING BLOOD SUGAR(GLUCOSE)	PHOTOMETRY	82.16	mg/dL

Bio. Ref. Interval. :-

As per ADA Guideline: Fasting Plasma Glucose (FPG)	
Normal	70 to 100 mg/dl
Prediabetes	100 mg/dl to 125 mg/dl
Diabetes	126 mg/dl or higher

Note :

The assay could be affected mildly and may result in anomalous values if serum samples have heterophilic antibodies, hemolyzed ,
icteric or lipemic. The concentration of Glucose in a given specimen may vary due to differences in assay methods, calibration and
reagent specificity. For diagnostic purposes results should always be assessed in conjunction with patients medical history, clinical
findings and other findings.

Please correlate with clinical conditions.

Method:- GOD-POD METHOD

Tests Done : AAROGRAM FULL HEALTH CHECKUP WITH
VITAMINS,ESR,PHOSPHOROUS

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd-NABL
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Patient Name : ANKIT(26Y/M)
Referred By : SELF
Home Collection : A-37 PRATIMA BHAWAN SECTOR 26 A BLOCK NOIDA NOIDA-01
NOIDA

Sample Collected on (SCT) : 29 Mar 2026 10:41
Sample Received on (SRT) : 29 Mar 2026 12:39
Report Released on (RRT) : 29 Mar 2026 15:05
Sample Type | Barcode : URINE | FI066500

TEST NAME	METHODOLOGY	VALUE	UNITS	Bio. Ref. Interval.
Complete Urinogram				
<u>Physical Examination</u>				
VOLUME	Visual Determination	3	mL	-
COLOUR	Visual Determination	PALE YELLOW	-	Pale Yellow
APPEARANCE	Visual Determination	CLEAR	-	Clear
SPECIFIC GRAVITY	pKa change	< 1.003	-	1.003-1.030
PH	pH indicator	5.5	-	5-8
<u>Chemical Examination</u>				
URINARY PROTEIN	PEI	ABSENT	mg/dL	Absent
URINARY GLUCOSE	GOD-POD	ABSENT	mg/dL	Absent
URINE KETONE	Nitroprusside	ABSENT	mg/dL	Absent
URINARY BILIRUBIN	Diazo coupling	ABSENT	mg/dL	Absent
UROBILINOGEN	Diazo coupling	Normal	mg/dL	<=0.2
BILE SALT	Hays sulphur	ABSENT	-	Absent
BILE PIGMENT	Ehrlich reaction	ABSENT	-	Absent
URINE BLOOD	Peroxidase reaction	ABSENT	-	Absent
NITRITE	Diazo coupling	ABSENT	-	Absent
LEUCOCYTE ESTERASE	Esterase reaction	ABSENT	-	Absent
<u>Microscopic Examination</u>				
MUCUS	Microscopy	ABSENT	-	Absent
RED BLOOD CELLS	Microscopy	ABSENT	cells/HPF	0-2
URINARY LEUCOCYTES (PUS CELLS)	Microscopy	ABSENT	cells/HPF	0-5
EPITHELIAL CELLS	Microscopy	ABSENT	cells/HPF	0-5
CASTS	Microscopy	ABSENT	-	Absent
CRYSTALS	Microscopy	ABSENT	-	Absent
BACTERIA	Microscopy	ABSENT	-	Absent
YEAST	Microscopy	ABSENT	-	Absent
PARASITE	Microscopy	ABSENT	-	Absent

(Reference : *PEI - Protein error of indicator, *GOD-POD - Glucose oxidase-peroxidase)

~~ End of report ~~

Tests Done : AAROGYAM FULL HEALTH CHECKUP WITH
VITAMINS,ESR,PHOSPHOROUS

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd



Dr.Sneha Singh
MD(Path)



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CONDITIONS OF REPORTING

- ✓ The reported results are for information and interpretation of the referring doctor only.
- ✓ It is presumed that the tests performed on the specimen belong to the patient; named or identified.
- ✓ Results of tests may vary from laboratory to laboratory and also in some parameters from time to time for the same patient.
- ✓ Should the results indicate an unexpected abnormality, the same should be reconfirmed.
- ✓ Only such medical professionals who understand reporting units, reference ranges and limitations of technologies should interpret results.
- ✓ This report is not valid for medico-legal purpose.
- ✓ Docon Technologies Private Limited, Thyrocare Technologies Limited and its employees/representatives do not assume any liability, responsibility for any loss or damage that may be incurred by any person as a result of presuming the meaning or contents of the report.

EXPLANATIONS

- ✓ **Name** - The name is as declared by the client and recorded by the personnel who collected the specimen.
- ✓ **Ref.By** - The name of the doctor who has recommended testing as declared by the client.
- ✓ **Labcode** - This is the accession number in our laboratory and it helps us in archiving and retrieving the data.
- ✓ **Barcode** - This is the specimen identity number and it states that the results are for the specimen bearing the barcode (irrespective of the name).
- ✓ **SCT** - Specimen Collection Time - The time when specimen was collected as declared by the client.
- ✓ **SRT** - Specimen Receiving Time - This time when the specimen reached our laboratory.
- ✓ **RRT** - Report Releasing Time - The time when our pathologist has released the values for Reporting.
- ✓ **Reference Range** - Means the range of values in which 95% of the normal population would fall.

SUGGESTIONS

- ✓ Values out of reference range requires reconfirmation before starting any medical treatment.
- ✓ Retesting is needed if you suspect any quality shortcomings.
- ✓ For suggestions, complaints or feedback, write to us at grievance-office@docon.co.in or call us on 7022000900.

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